

CUSTOMER	:	IDAMAN PHARMA MANUFACTURING SDN BHD
DESCRIPTION	:	DEXTROMETHORPHAN
MEASUREMENT	:	W:210mm X H:200mm
MATERIAL	:	SMPO 060

SHEET SIZE	:	NIL	COLOUR :	Black
PROCESS	:	NIL		
FINISHING	:	UNFOLD		
BOARD OUTING	:	NIL		

MATERIAL CODE:	Attn : MS. ZAKIAH From : MALA I hereby verify & confirm that this draft is the final copy. Any subsequent changes of the design and content of the end product will not be borne by THUNDER PRINT SDN. BHD.
Lt-311.00	
MOULD NO : -	Customer Signature & Date

27.10.2022

pharmaniaga®

Dextromethorphan

 **Compound**

DESCRIPTION

Brown syrup.

COMPOSITION

Each 5 mL contains:

Dextromethorphan hydrobromide 15 mg.

Promethazine hydrochloride 4 mg.

Preservative: Sodium benzoate 0.10% w/v.

ACTIONS

Promethazine is an H₁ receptor blocking agent, which among other effects, provides a drying effect on the nasal and oral mucosa. In addition to its antihistaminic action, it provides clinically useful sedative and antiemetic effects. In therapeutic doses promethazine produces no significant effects on the cardiovascular system.

Dextromethorphan is an antitussive agent. It acts centrally and elevates the threshold for coughing. It is about equal to codeine in depressing the cough reflex.

Promethazine is well absorbed from gastrointestinal tract. Clinical effects are apparent within 20 minutes after oral administration and generally last for 4 to 6 hours, although they may persist as long as 12 hours. Promethazine is metabolised by the liver to a variety of compounds; the sul-

foxides of promethazine and N-demethylpromethazine are the predominant metabolites appearing in the urine.

Dextromethorphan is rapidly absorbed from the gastro-intestinal tract and exerts its effects in 15 to 30 minutes. The duration of action after oral administration is approximately 3 to 6 hours. Dextromethorphan is metabolised primarily by liver enzymes undergoing O-demethylation, N-demethylation and partial conjugation with glucuronic acid and sulphate. In humans, (+)-3-hydroxy-N-methylmorphinan, and traces of unmetabolised drug were found in urine after oral administration.

INDICATIONS

Dextromethorphan Compound is indicated for the temporary relief of coughs and upper respiratory symptoms associated with allergy or common cold.

CONTRAINDICATIONS

This medication is contraindicated in individuals known to be hypersensitive or to have had an idiosyncratic reaction to promethazine or to other phenothiazines. Dextromethorphan Compound contains an antihistamine which is also contraindicated for use in the treatment of lower respiratory tract symptoms, including asthma.

Dextromethorphan Compound should not be used in patients receiving monoamine oxidase inhibitor (MAOI) therapy.

Dextromethorphan Compound is also not recommended in children below 2 years of age.

ADVERSE REACTIONS

Promethazine side-effects include drowsiness, thickening of mucus: rarely, blurred vision or any change in vision, confusion, difficult or painful urination, dizziness, dryness of mouth, nose or throat, hypotension, increased sweating, loss of appetite, photosensitivity, ringing or buzzing in ears, skin rash and tachycardia.

Dextromethorphan occasionally causes slight drowsiness, dizziness and gastro-intestinal disturbances.

WARNINGS

This preparation contains sodium metabisulphite, that may cause serious allergic-type reactions in certain susceptible people. Do not use if known to be hypersensitive to bisulphites.

This product contains Promethazine hydrochloride. It should not be used in paediatric patients less than 2 years of age because of the potential for fatal respiratory depression. Use carefully and follow the advice from doctor or pharmacist for children aged 2 to 6 years.

This medication may cause marked drowsiness or impair the mental and/or physical abilities required for the performance of potentially hazardous tasks, such as driving a vehicle or operating machinery. Patients should be told to avoid engaging in such activities until it is known that they do not become drowsy or dizzy. Children should be supervised to avoid potential harm in bike riding or in other hazardous activities.

The concomitant use of alcohol or other central nervous system depressants, including narcotic analgesics, sedatives, hypnotics and tranquilizers, may have an additive effects and should be avoided or their dosage reduced.

PRECAUTIONS

Promethazine may lower seizure threshold. This should be taken into consideration when administering to persons with known seizure disorders or when giving in combination with narcotics or local anaesthetics which may also affect seizure threshold.

Caution is advised in patients with a history of sleep apnoea, in patients with stenosing peptic ulcer, and in pyloroduodenal obstruction.

Dextromethorphan should be used with caution in atopic children.

Pregnancy

Adequate and well controlled studies in humans have not been done for promethazine and dextromethorphan.

For promethazine, it has been reported that administration within 2 weeks prior to delivery may inhibit platelet aggregation in the newborn.

This medication should be used during pregnancy only if the potential benefit justifies the potential risk to the foetus.

Breast-feeding

Small amounts of antihistamines may be distributed into breast milk causing adverse effects such as unusual excitement or irritability in infants.

Some studies have indicated that the use of promethazine in children up to 2 years of age may be associated with the sudden infant death syndrome and an increase in sleep apnoea, thus, possibly increasing the risk to the nursing infant.

Antihistamines may also inhibit lactation.

Therefore, use is not recommended in nursing mothers.

Paediatrics

Use is not recommended in children under 2 years of age due to the CNS side-effects and an increased tendency towards convulsions caused by promethazine.

Geriatrics

Dizziness, sedation, confusion and hypotension may be more likely to occur in geriatric patients taking antihistamines. The elderly are especially susceptible to the anticholinergic side-effects of promethazine such as dryness of the mouth and urinary retention (especially in males). If these side-effects occur and continue or are severe, the medication should be discontinued.

DRUG INTERACTIONS

Alcohol or other CNS depressants:

Concurrent use may potentiate the CNS depressant effects of these medications or antihistamines.

Anticholinergics:

Anticholinergic effects may be potentiated when medications are used concurrently.

Epinephrine:

Alpha-adrenergic effects of epinephrine may be blocked possibly resulting in severe hypotension and tachycardia.

Levodopa:

Antiparkinsonian effect of levodopa may be inhibited when used concurrently with phenothiazine derivatives.

Monoamine oxidase inhibitors:

Concurrent use with antihistamines may prolong and intensify the anticholinergic and CNS depressant effects of antihistamines as well as increase the risk of hypotension and extrapyramidal reactions. Concurrent use with dextromethorphan may cause adrenergic crisis, collapse, coma; concurrent use is contraindicated.

DOSAGE AND ADMINISTRATION

Adults : 10 mL every six to eight hours.
Maximum of 40 mL daily.

Children :

6-12 years : 5 mL every six to eight hours.
Maximum of 20 mL daily.
2- 6 years : 2.5 mL every six to eight hours.
Maximum 10 mL daily.
Children under 2 years: Use is not recommended.

ROUTE OF ADMINISTRATION

Oral

OVERDOSAGE

Symptoms:

Promethazine:

Range from mild depression of the central nervous system

and cardiovascular system to profound hypotension, respiratory depression and unconsciousness. Stimulation may be evident, especially in children and geriatrics. Convulsions may rarely occur. A paradoxical reaction has been reported in children characterised by hyperexcitability and nightmares. Atropine-like signs and symptoms i.e. dry mouth, fixed and dilated pupils, flushing as well as gastrointestinal symptoms may occur.

Dextromethorphan:

Central excitement and mental confusion. Very high doses may produce respiratory depression.

Treatment:

Treatment of overdosage is essentially symptomatic and supportive. Only in cases of extreme overdosage or individual sensitivity do vital signs including respiration, pulse, blood pressure, temperature and EKG need to be monitored.

Activated charcoal orally or by lavage may be given, or sodium or magnesium sulphate orally as a cathartic. Attention should be given to the re-establishment of adequate respiratory exchange through provision of a patient airway and institution of assisted or controlled ventilation. Diazepam may be used to control convulsions. Acidosis and electrolyte losses should be corrected.

The antidotal efficacy of narcotic antagonists to dextromethorphan has not been established; note that any of the depressant effects of promethazine are not reversed by naloxone. Avoid analeptics which may cause convulsion. Severe hypotension usually responds to the administration of norepinephrine or phenylephrine. Epinephrine should not be used since its use in a patient with partial adrenergic blockade may further lower the blood pressure. Limited experience with dialysis indicates that it is not helpful.

STORAGE CONDITIONS

Store below 30°C.
Keep container tightly closed.
Protect from light.

SHELF LIFE

Product should not be used beyond the expiry date imprinted on the product packaging.

PRESENTATION

In bottle of 120 mL.

Date of revision: 21st September 2022

PRODUCT REGISTRATION HOLDER:

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